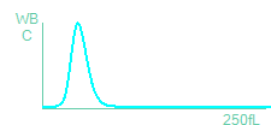
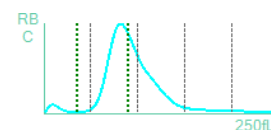


	Lab ID : 13905	Collected On : 08/06/2026 19:22:47
Name : MR. VINOD CHORGE	Age/Sex : 33 Yrs. / M	Received On : 08/06/2026 19:23:32
Ref. By : Self		Reported On : 08/06/2026 20:16:47
Sent By : NIDAN PATHOLOGY LAB & DIAGNOSTIC		Printed : 10/06/2026 11:29:28

COMPLETE BLOOD COUNT

Test	Result	Unit	Reference Range
HAEMOGLOBIN	: 13.30	g/dl	13.0-17.0 g/dl
Method:Coulter principle- photometric measurement			
R.B.C COUNT	: 5.23	$\times 10^6/\mu\text{L}$	4.5-5.9 $\times 10^6/\mu\text{L}$
Method:Colter Principle			
PACK CELL VOLUME	: 46.70	%	37-53 %
Method:Calculated			
MCV	: 89.29	fL	80-100 fL
Method:Calculated			
MCH	: 25.43	Pg	25-35 Pg
Method:Calculated			
MCHC	: 28.50	g/dL	25-35 g/dL
RDW-CV	: 15.20	%	11-15 %
Method:Derived from histogram			
RDW-SD	: 42.10	fL	39-46 fL
Method:Derived from histogram			
Mentzer Index	: 17.07		<13.0 Suspected Hemoglobinopathy
Method:Calculated			



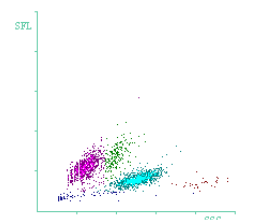
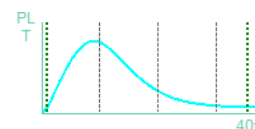
WBC PARAMETERS

W.B.C. TOTAL COUNT	: 7.91	$\times 10^3/\mu\text{L}$	4-11 $\times 10^3/\mu\text{L}$
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Method:Coulter principle

DIFFERENTIAL COUNT .(W.B.C.)

NEUTROPHILS %	: 52.50	%	40 - 70 %
Method:VCSn Technology			
LYMPHOCYTES %	: 37.00	%	25 - 45 %
Method:VCSn Technology			
EOSINOPHILS %	: 1.40	%	1 - 6 %
Method:VCSn Technology			
MONOCYTES %	: 9.00	%	2 - 10 %
Method:VCSn Technology			
BASOPHILS %	: 0.10	%	0-1 %
Method:VCSn Technology			



	Lab ID : 13905	Collected On : 08/06/2026 19:22:47
13905 080626 Name : MR. VINOD CHORGE	Age/Sex : 33 Yrs. / M	Received On : 08/06/2026 19:23:32
Ref. By : Self		Reported On : 08/06/2026 20:16:47
Sent By : NIDAN PATHOLOGY LAB & DIAGNOSTIC		Printed : 10/06/2026 11:29:28

ABSOLUTE COUNTS

ABSOLUTE NEUTROPHILS	:	4153	/c.mm	2000-7000 /c.mm
ABSOLUTE LYMPHOCYTES	:	2927	/c.mm	1000-3000 /c.mm
ABSOLUTE EOSINOPHILS	:	111	/c.mm	20-500 /c.mm
ABSOLUTE MONOCYTES	:	712	/c.mm	200-1000 /c.mm
ABSOLUTE BASOPHILS	:	8	/c.mm	20-100 /c.mm

PLATELET COUNT

PLATELET COUNT	:	232.00	x10 ³ /uL	150-450 x10 ³ /uL
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Method:Coulter principle

MPV	:	9.40	fL	6.50-12.0 fL
PCT	:	0.22	%	0.22 - 0.24 %
PDW	:	14.50	%	11- 28 %

Sample Type: EDTA(Whole Blood)

----- End Of Report -----



	Lab ID : 13905	Collected On : 08/06/2026 19:22:47
13905 080626 Name : MR. VINOD CHORGE	Age/Sex : 33 Yrs. / M	Received On : 08/06/2026 19:23:32
Ref. By : Self		Reported On : 09/06/2026 14:06:32
Sent By : NIDAN PATHOLOGY LAB & DIAGNOSTIC		Printed : 10/06/2026 11:29:28

ERYTHROCYTE SEDIMENTATION RATE (ESR)

Test	Result	Unit	Biological Reference Interval
ERYTHROCYTES SEDIMENTATION RATE	12	mm at 1hr	0-15 mm at 1hr

Westergren's method

Interpretation:

High ESR is not diagnostics of any disease but just indicative of some inflammatory process. ESR is to be used to monitor outcome of therapy. Microcytic anemia can increase ESR. High ESR can also be seen in apparently healthy adults.

Sample Type:EDTA

----- End Of Report -----



	Lab ID : 13905	Collected On : 08/06/2026 19:22:47
13905 080626 Name : MR. VINOD CHORGE	Age/Sex : 33 Yrs. / M	Received On : 08/06/2026 19:23:32
Ref. By : Self		Reported On : 08/06/2026 21:33:09
Sent By : NIDAN PATHOLOGY LAB & DIAGNOSTIC		Printed : 10/06/2026 11:29:28

LIVER FUNCTION TEST (LFT)

Test	Result	Unit	Biological Reference Interval
TOTAL BILIRUBIN Method: Jendrassik Grof	: 0.26	mg/dl	0-1.2 mg/dl
DIRECT BILIRUBIN Method: Jendrassik Grof	: 0.10	mg/dl	0-0.4 mg/dl
INDIRECT BILIRUBIN Calculated	: 0.16	mg/dl	0-0.7 mg/dl
S. G. O.T Method :IFCC	: 46.08	IU/L	05-34 IU/L
S. G. P. T Method :IFCC	: 87.46	IU/L	0-55 IU/L
ALKALINE PHOSPHATASE Method :IFCC p-NPP/AMP	: 104.93	IU/L	40-150 IU/L
TOTAL PROTEINS Method :Biuret Method	: 7.56	g/dl	6.4-8.2 g/dl
ALBUMIN Method: Bromocresol Green	: 5.19	g/dl	3.4-5.0 g/dl
GLOBULINS	: 2.37	g/dl	2.0-4.1 g/dl
A/G RATIO	: 2.18		1.0 -2.1

Note :

1. In an asymptomatic patient, Non alcoholic fatty liver disease (NAFLD) is the most common cause of increased AST, ALT levels. NAFLD is considered as hepatic manifestation of metabolic syndrome.
2. In most type of liver disease, ALT activity is higher than that of AST; exception may be seen in Alcoholic Hepatitis, Hepatic Cirrhosis, and Liver neoplasia. In a patient with Chronic liver disease, AST:ALT ratio>1 is highly suggestive of advanced liver fibrosis.
3. In known cases of Chronic Liver disease due to Viral Hepatitis B & C, Alcoholic liver disease or NAFLD, Enhanced liver fibrosis (ELF) test may be used to evaluate liver fibrosis.
4. In a patient with Chronic Liver disease, AFP and Des-gamma carboxyprothrombin (DCP)/PIVKA II can be used to assess risk for development of Hepatocellular Carcinoma.

----- End Of Report -----



	Lab ID : 13905	Collected On : 08/06/2026 19:22:47
13905 080626 Name : MR. VINOD CHORGE	Age/Sex : 33 Yrs. / M	Received On : 08/06/2026 19:23:32
Ref. By : Self		Reported On : 08/06/2026 21:41:14
Sent By : NIDAN PATHOLOGY LAB & DIAGNOSTIC		Printed : 10/06/2026 11:29:28

RENAL PROFILE/KIDNEY FUNCTION TEST (RFT/KFT) WITHOUT ELECTROLYTE

Test	Result	Unit	Biological Reference Interval
BLOOD UREA	: 19.46	mg/dl	15-38 mg/dl
GLDH Method			
BLOOD UREA - NITROGEN	: 9.09	mg/dl	7-18 mg/dl
Calculated			
SERUM CREATININE	: 0.85	mg/dl	Male- 0.70-1.30 mg/dL Female- 0.55-1.02 mg/dL
Method: Alkaline picrate			
SERUM URIC ACID	: 5.71	mg/dl	3.5-7.2 mg/dl
Method : Uricase-Colorimetric			
SERUM CALCIUM	: 9.36	mg/dl	8.5-10.2 mg/dl
Arsenazo			
SERUM PHOSPHORUS	: 4.30	mg/dl	2.6-4.7 mg/dl
Method: Phosphomolybdate			
TOTAL PROTEINS	: 7.56	g/dl	6.4-8.2 g/dl
Method : Biuret			
ALBUMIN	: 5.19	g/dl	3.4-5.0 g/dl
Method: Bromocresol Green			
GLOBULINS	: 2.37	g/dl	2.0-4.1 g/dl
Calculated			
A/G RATIO	: 2.19		1.0 -2.1

----- End Of Report -----



	Lab ID : 13905	Collected On : 08/06/2026 19:22:47
13905 080626 Name : MR. VINOD CHORGE	Age/Sex : 33 Yrs. / M	Received On : 08/06/2026 19:23:32
Ref. By : Self		Reported On : 08/06/2026 21:41:19
Sent By : NIDAN PATHOLOGY LAB & DIAGNOSTIC		Printed : 10/06/2026 11:29:28

LIPID PROFILE

Test	Result	Unit	Reference Range
TOTAL CHOLESTEROL	: 173.20	mg/dl	Desirable : < 200 Borderline high : 201 - 240 High : > 240
CHOD-PAP			
S. TRIGLYCERIDES	: 107.31	mg/dl	Desirable : < 150 mg/dl Borderline high : 151 - 199 mg/dl High : 200 - 499 Very High : >500 mg/dl
Enzymatic, end point coupled assay			
HDL CHOLESTEROL	: 43.69	mg/dl	Low :< 40 High : > 60
Acclerator selective detergent			
LDL CHOLESTEROL	: 108.05	mg/dl	Optimal : <100 Near / Above Optimal : 101 - 129 Borderline High : 130 - 159 High : > 160
NON-HDL CHOLESTROL	: 129.51	mg/dL	<130 mg/dL
(Calculated)			
VLDL CHOLESTEROL (Calculated)	: 21.46	mg/dl	Below 30 mg/dl
TOTAL CHOL/HDL CHOL RATIO	: 3.96		Desirable/Low Risk : 3.3 - 4.4 Borderline/Middle Risk : 4.5 - 7.1 Elevated/High Risk : 7.2 - 11.0
(Calculated)			
LDL CHOL / HDL CHOL RATIO	: 2.5		Desirable/Low Risk : 0.5 - 3.0 Borderline/Middle Risk : 3.1 - 6.0 Elevated/High Risk : >6.1
(Calculated)			

----- End Of Report -----



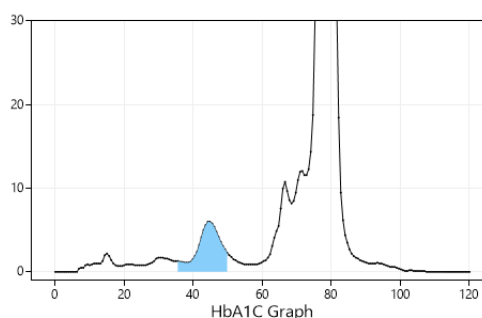
	Lab ID : 13905	Collected On : 08/06/2026 19:22:47
Name : MR. VINOD CHORGE	Age/Sex : 33 Yrs. / M	Received On : 08/06/2026 19:23:32
Ref. By : Self		Reported On : 08/06/2026 20:16:48
Sent By : NIDAN PATHOLOGY LAB & DIAGNOSTIC		Printed : 10/06/2026 11:29:28

HBA1C (GLYCATED HEMOGLOBIN)

Test	Result	Unit	Reference Range
GLYCOSYLATED HAEMOGLOBIN (HbA1c)	7.60	%	Normal : <5.7 % Prediabetic : 5.8 - 6.5 % Diabetic : >6.5 %
ESTIMATED AVERAGE GLUCOSE	171.42	mg/dl	
Method	HPLC		

Interpretation :

- HbA1c is used for monitoring diabetic control. It reflects the estimated Average Glucose (eAG) over three months.
- Trends in HbA1c are a better indicator of diabetic control than a solitary test.
- HbA1c Estimation can get affected in Anemia, Chronic renal failure.
- HbA1c is falsely low in diabetics with hemolytic disease. Fructosamine is recommended in these patients which indicates diabetics control over 15 days.



Peak	Time	ABS	Area	Result
HbA1a	14	9	200	0.2
HbA1b	19	22	780	0.9
HbF	26	9	170	0.2
LA1c+	36	17	1220	1.4
HbA1c	50	60	6440	7.6
HbA0	87	1519	82360	90.3
V_win	-	-	-	-

Total Area : 91170.0
HbA1c(IFCC) : 59.5 mmol/mol
eAG (mmol/l) : 09.5 mmol/l
eAG (mg/dl) : 171.2 mg/dl
HbA1c(NGSP) : 7.6%
Reference Range : 4.5% - 6.5%



	Lab ID : 13905	Collected On : 08/06/2026 19:22:47
Name : MR. VINOD CHORGE	Age/Sex : 33 Yrs. / M	Received On : 08/06/2026 19:23:32
Ref. By : Self		Reported On : 08/06/2026 21:11:03
Sent By : NIDAN PATHOLOGY LAB & DIAGNOSTIC		Printed : 10/06/2026 11:29:28

THYROID PROFILE TOTAL (T3, T4, TSH)

Test	Result	Unit	Biological Reference Interval
TOTAL T3 (Method : CLIA)	: 114.07	ng/dl	60-181 ng/dl
TOTAL T4 (Method : CLIA)	: 6.22	ug/dl	Hypothyroid -0.0- 4.5 Euthyroid- 4.5 - 14.5 Hyperthyroid -14.5-19.1 Pregnant Euthyroid -6.4- 10.7 Cord Blood (0 day) -7.4 - 13.0 Neonatal (1-4 days) -14.0 - 28.4
SERUM T.S.H. Ultrasensitive (Method : CLIA)	: 2.114	uIU/ml	0.28-5.50 First Trimester : 0.33-4.59 Second Trimester : 0.35-4.10 Third trimester : 0.21-3.15

REFERENCE RANGES :

AGE	T3	T4	TSH
0-3 days	96 - 292 ng/dl	5.37 - 22.4 ug/dl	5.17 - 14.6 uIU/ml
4-30 days	60 - 243 ng/dl	5.24 - 23.2 ug/dl	0.43 - 16.1 uIU/ml
1 - 12 mth	81 - 281 ng/dl	5.37 - 16.0 ug/dl	0.62 - 8.05 uIU/ml
1 - 6 yrs	83 - 252 ng/dl	5.26 - 14.8 ug/dl	0.54 - 4.53 uIU/ml
7 - 11 yrs	92 - 219 ng/dl	5.70 - 14.1 ug/dl	0.66 - 4.14 uIU/ml

INTERPRETATION :

TSH	T3	T4	INTERPRETATION
High	Normal	Normal	Mild (Sub Clinical) Hypothyroidism
High	Low or Normal	Low	Hypothyroidism
Low	Normal	Normal	Mild (Subclinical) Hyperthyroidism
Low	High or normal	High or Normal	Hyperthyroidism
Low	Low or Normal	Low or Normal	Non thyroid illness, Pituitary (secondary) hypothyroidism

Note :

In patients on therapy only TSH levels may be misleading, instead free T3 and free T4 levels may be helpful. Clinical diagnosis should not be made on the findings of a single test result, but should integrate both clinical and laboratory data.

----- End Of Report -----



	Lab ID : 13905	Collected On : 08/06/2026 19:22:47
13905 080626 Name : MR. VINOD CHORGE	Age/Sex : 33 Yrs. / M	Received On : 08/06/2026 19:23:32
Ref. By : Self		Reported On : 08/06/2026 21:11:04
Sent By : NIDAN PATHOLOGY LAB & DIAGNOSTIC		Printed : 10/06/2026 11:29:28

25-HYDROXY VITAMIN D (D2+D3)

Test	Result	Unit	Biological Reference Interval
25-HYDROXY VITAMIN D (D2+D3) (Method : CLIA)	: 9.41	ng/ml	Deficiency : <20 Insufficiency : 20–30 Sufficiency : 30–100 Toxicity : >100

Interpretation:

Vitamin D is a fat soluble vitamin and exists in two main forms as cholecalciferol (vitamin D3) which is synthesized in skin from 7-dehydrocholesterol in response to sunlight exposure and Ergocalciferol (vitamin D2) present mainly in dietary sources. Both cholecalciferol are converted to 25 (OH) vitamin D in liver.

Testing for 25 (OH) vitamin D is recommended as it is the best indicator of vitamin D nutritional status as obtained from sunlight exposure and dietary intake. For diagnosis of vitamin D deficiency it is recommended to have clinical correlation with serum 25 (OH) vitamin D, serum calcium, serum PTH and serum alkaline phosphatase.

During monitoring of oral vitamin D therapy-suggested testing of serum 25(OH) vitamin D is after 12 weeks or 3 months of treatment. However, the required dosage of vitamin D supplements and time to achieve sufficient vitamin D levels show significant seasonal (especially winter) & individual variability depending on age, body fat, sun exposure, physical activity, genetic factors (especially variable vitamin D receptor response), associated liver or renal disease, malabsorption syndromes and calcium or magnesium deficiency influencing the vitamin D metabolism vitamin D toxicity is known but very rare. Kindly correlate clinically, repeat with fresh sample if indicated.

----- End Of Report -----



	Lab ID : 13905	Collected On : 08/06/2026 19:22:47
13905 080626 Name : MR. VINOD CHARGE	Age/Sex : 33 Yrs. / M	Received On : 08/06/2026 19:23:32
Ref. By : Self		Reported On : 09/06/2026 14:08:48
Sent By : NIDAN PATHOLOGY LAB & DIAGNOSTIC		Printed : 10/06/2026 11:29:28

VITAMIN B12

Test	Result	Unit	Biological Reference Interval
VITAMIN B12 (Method : CLIA)	: 241.00	pg/ml	120-914 pg/ml

SIGNIFICANCE :

Vitamin B 12 is a water soluble vitamin. It's important sources are animal products and hence pure vegans are prone to deficiency. Vitamin B12 deficiency may also develop as a result of lack of intrinsic factor, gastric surgery and intestinal conditions like Crohn's disease and celiac disease. Its deficiency can cause megaloblastic anaemia, subacute combined degeneration of spinal cord , sensoromotor deficiency and even dementia.

----- End Of Report -----



	Lab ID : 13905	Collected On : 08/06/2026 19:22:47
13905 080626 Name : MR. VINOD CHORGE	Age/Sex : 33 Yrs. / M	Received On : 08/06/2026 19:23:32
Ref. By : Self		Reported On : 09/06/2026 14:09:14
Sent By : NIDAN PATHOLOGY LAB & DIAGNOSTIC		Printed : 10/06/2026 11:29:28

IRON STUDIES (IRON + TIBC)

Test	Result	Unit	Biological Reference Interval
SERUM IRON	: 58.00	ug/dl	50-170 ug/dl
Method: ferene			
SERUM UIBC	: 284.00	ug/dl	120-470 ug/dl
Method: Photometric test using Ferene			
SERUM TIBC	: 342.00	ug/dl	250-450 ug/dl
Calculated			
Transferrin Saturation	: 16.96	%	14-50 %
Calculated			

Interpretation :

1. Measurement of serum Iron, TIBC and the percentage of iron saturation of transferrin are useful screening tests for iron deficiency anemia.
2. However , serum iron exhibits significant diurnal variation and may transiently rise or reach reference values after dietary or iron supplements and post blood transfusion.
3. The diagnostics specificity of low serum iron for iron deficiency is lost in the presence of acute and chronic inflammatory processes.

Test	Iron deficiency Anaemia	Anaemia of Chronic Disease	Iron Overload
S. Iron	Decreased	Decreased	Increased
TIBC	Increased	Decreased or Normal	Increased or Normal
% Transferrin Saturation	Decreased	Decreased or Normal	Increased or Normal
S. Ferritin	Decreased	Increased	Increased or Normal

----- End Of Report -----



	Lab ID : 13905	Collected On : 08/06/2026 19:22:47
13905 080626 Name : MR. VINOD CHORGE	Age/Sex : 33 Yrs. / M	Received On : 08/06/2026 19:23:32
Ref. By : Self		Reported On : 09/06/2026 14:10:36
Sent By : NIDAN PATHOLOGY LAB & DIAGNOSTIC		Printed : 10/06/2026 11:29:28

ESTIMATED GLOMERULAR FILTRATION RATE (EGFR)

Test	Result	Unit	Reference Range
AGE	: 33		
SERUM CREATININE	: 0.85	mg/dl	Male- 0.70-1.30 mg/dL Female- 0.55-1.02 mg/dL
Method: Alkaline picrate			
EGFR	: 118	ml/min/1.73m ²	90 and above : Normal 55 - 89 Mild decrease 35-54 Moderate decrease 15 - 34 : Severe decrease

----- End Of Report -----

